False, indisputable

False/inaccurate, disputable

OFFICIAL

False, because Gie City lied/withheld information

Subjective - projecting their values onto Gie City's life

Government of South Australia
Mental Health Directorate
Central Adelaide Local Health Network

Eastern Community Mental Health

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22/09/2016

Dear Dr.

RE: Giles Cullity, DOB: 22/07/1993

I see him again at Tranmere clinic. After citalopram is increased to 30mg, it was reported that he became more animated by other. However, he did not feel so. He constantly minimises effects of medication and does not feel he has mental illnesses or need for treatment.

He does not engage well with psychosocial rehabilitation programs or case management with Mental Health Service. We will continue seeing him for the time being.

Regards,

Dr. Catherine Ye

Senior Consultant Psychiatrist

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25 October 2016

Supreme Court of South Australia
241-259 Victoria Square
ADELAIDE SA 5000

For the attention of the Presiding Officer
The Honourable Justice — P Kelly

PSYCHIATRIC REPORT

RE: **Giles CULLITY**
DOB: **22/07/1993**
COURT FILE: **SCCRM-16-257**

Thank you for your request for a psychiatric report for the Court to consider Mr Cullity's application for variation of a condition of a Bond under the Criminal Law Consolidation Act. This report is based on direct interview with him on 20 October 2016 and a review of the case notes of Mental Health Services.

I am a legally qualified medical practitioner, registered in South Australia as a specialist psychiatrist. I obtained my basic medical qualifications and Master Degree of Medicine from Shanghai Medical University, China, in 1995 and 2000, respectively. I gained Fellowship of The Royal Australian and New Zealand College of Psychiatrists in 2010 and have worked as a consultant psychiatrist since. I worked at Modbury

Hospital psychiatric ward from 2010 till 2013 before I was appointed as Acting Clinical Lead of Inpatient Rehabilitation Service, Glenside Health Service in late 2013. Since February 2016, I worked for 2 months in Eastern Acute Inpatient Unit, Glenside Campus and currently I am working at Eastern Community Mental Health Service, Hallett Team.

I took Mr Cullity's care over in May 2016 and have reviewed him a number of times. For the purpose of this report, I reviewed him again on 20 October 2016.

I am grateful for the court providing me with relevant documentation including:

- Report request form.
- Transcripts of court process.
- Application for variation of a condition of a bond.
- Court sentence transcription on 11 December 2014.
- Forensic psychiatric assessment dated on 2 August 2013 by Dr Craig Raeside
- Addendum psychiatric report dated on 23 October 2013 by Dr Craig Raeside.
- Psychiatric report dated on 11 January 2014 by Dr Lynette Rose.
- CBIS entry and OACIS discharge summaries.

Synopsis and Background History

Mr Cullity is a 23 year old single man who has been residing with his parents as a condition of the Bond. His main source of income is the Disability Support Pension. He has been diagnosed with Schizophrenia. It is likely he has a prolonged prodromal psychosis or untreated psychosis before this diagnosis was fully established. When he offended, he was untreated.

Mr Cullity started to contact Mental Health Services when he was in high school. At that time, he presented with depression and suicidal ideation. On reflection, he started to experience unusual thoughts back then. Mr Cullity became increasingly grandiose about his computer project and he was more preoccupied with his so called genetic illness. Meanwhile, his academic performance deteriorated as well as other daily functions including personal hygiene.

He has been admitted to hospital a couple of times and currently is followed up by Eastern Community Mental Health Service. In the past, he was under a Community Treatment Order due to non-compliance

issues. He is still under supervision of his parents for oral antipsychotic and antidepressant medication. Mr Cullity has chronic delusions around his computer project and appearance. The intensity of his delusions fluctuates with treatment.

He appears lacking of insight into his illness and the need for ongoing treatment. Constantly, he debates with the treating clinicians about his diagnosis. He is attending the medical appointments as a part of the forensic condition. He remains reluctant to continue treatment after the Bond finishes in the future.

Forensic history:

- Assault police in 2010
- Attempted murder in 2013

Since the first contact with the Mental Health Service, Mr Cullity has been treated with olanzapine, dose varied from 10mg to 15mg. Citalopram was started for depression. Dose range was 30-40mg. In January 2014, non-compliance was reported. Therefore depot was started and CTO L2 was put in place. From February 2014 to September 2015, he was on paliperidone 75mg 4 weekly and the dose was reduced to 50mg until February 2015 when it was further reduced to 25mg. Until February 2016, depot was ceased, risperidone 2mg was started. Due to his somatic complaints, which however, was likely secondary to his somatic delusion, risperidone was changed to aripiprazole. The dose was titrated up to 20mg. He started to complain about akathisia. Currently, he is on 15mg.

Developmental and Social History

Mr Cullity is the second of 4 boys. He was born in Scotland and came to Australia with family at the age of eight. He reports no problems in his early life. His father works as a professor of Philosophy of University of Adelaide and his mother is a primary school teacher of a private school.

He attended St. Ignatius College for primary school. He does not like that school. He went to Marryatville for high school where he was involved in the band. He did not have many friends in school and never had a girlfriend. His grades started to drop in Year 10 when he became obsessive about weight and muscles. He failed subjects in Year 10 and 11 and later some in Year 12. He managed to enter university by doing STAT but failed first year of bridging course.

He has no other interests or hobbies, other than his project.

Family history

Mood disorder was reported in his paternal grandfather, a paternal uncle and a maternal uncle.
His younger brother was diagnosed with Bipolar illness and the youngest brother has Asperger's syndrome.

Current Assessment

Mr Cullity is aware of the purpose of his assessment. He wants to know whether I will agree with him to move out.

He tells me that he has contemplated moving out for a couple of months as he believes it is possible for him financially. He starts to receive the Disability Support Pension this year and discussed this plan with his parents in July. He says his parents prefer him to live at home as they think that he cannot support himself adequately on the Disability Support Pension and they are also worried about lack of supervision over his compliance. Mr Cullity reveals that currently he is supervised by his parents 4 out of 7 days per week via skype for oral medication compliance. This is the first time he tells me that the supervision is done remotely.

He is keen to finish the computer project which he has been preoccupied with over the last few years. If he could move out, he feels he could perhaps speed up with the progress on his project and also focus on study. Mr Cullity tells me he recently enrolled in an online programming course. However, he will not get any qualification after the course and it is irrelevant to the project as there is no training for the project.

When I ask him whether it is likely he overestimates his ability and his preoccupation with the computer project is consistent with the diagnosis of schizophrenia. He disagrees. He thinks that he has cyclothymia, not bipolar or schizophrenia. He does not know what is meant by "formally diagnosed" although he has received a previous psychiatric assessment and formal reports which in the past he refused to provide to the Mental Health Service as documented in the file. Mr Cullity further adds that he cannot disbelieve what he believes as correct. He further studies DSM diagnostic criteria and he does not have delusions or hallucinations.

When we discuss the incident on that night, Mr Cullity admits that he stopped thinking about it one month after that. He is not sure how he feels about the victim or the whole incident. I asked him whether he feels

sorry for the victim. He answers yes, but he does not know the reason for his sorry. "I know that guy has schizophrenia too which was exacerbated, he has nerve damage and PTSD". Then Mr Cullity describes me the incident to me again. He recalls that the only thing he could do was to stab him, otherwise he would be punched. I prompt him again, if next time he was in the similar confrontational situation, what would he do? He answers, "not stabbing, perhaps walking away, I suppose".

Mr Cullity tells me that in 2013 his plan was to live in a car initially. However, he ended up living in streets. He thought he could start living in streets then purchase a car. He did not want to live with his parents in the family home as they had a lot of rules, like getting up at the certain time. He planned to finish his project in one year and he could then make money. At that time, buying a house for himself could be achievable. Currently he estimates that he might take 2-3 years to finish the project which potentially is still profitable. Once he programs to a certain stage, he would recruit a team to work for him. He believes his parents will not support his plan.

Mr Cullity is also fixated on perceived endocrinal diseases. He has had a number of tests which came back negative. However, he firmly believes that he has testosterone related illness. The intensity of his belief is consistent with delusion.

Mr Cullity repeatedly states that he does not believe he has schizophrenia and it is not fair to diagnose him after he stabbed someone. He shows reluctance to continue treatment if there is no court order in place. He asks me why there are so many people stabbing others out there but not being diagnosed with schizophrenia. I ask him to focus on his own case. If he did not have any mental disorders, where would he be after committing a crime? He says "jail". "Why can you live at home with your parents, instead of jail"? He could not answer that, but still debates if not argues with me about his diagnosis. He also says that none of the medications will help, because he has no hallucinations or delusions or negative symptoms. He says his lack of motivation is caused by medication instead of negative symptoms of schizophrenia.

Current Medication

- Aripiprazole 15mg mane
- Citalopram 30mg mane

Mental State Examination

Mr Cullity is young man without clear evidence of muscular deformity. He has proper masculine looks as opposed to his own perception. He could establish eye contact, but tended to avoid. He was quite blunt despite the grandiosity of his beliefs. At some stage, he became somewhat annoyed by the idea that he has a mental illness. Mr Cullity has a well systematized delusional system around his computer project and physical illness. Many of his thoughts are superficial, are plausible but cannot be further tested. He did not present with hallucinatory behaviours. His cognition was grossly intact. Mr Cullity has no insight into his mental illness and his overall judgement has been compromised.

Discussion with Mr Cullity's Parents

Mr Cullity's father was contacted. Professor Cullity expressed the family's concern over Mr Cullity's illness and his ability to live independently. Over years, his symptoms, especially his psychosis, does not seem to improve. He has ongoing delusions about the computer project and his physical appearance. He has no interests at all and is very withdrawn socially. His social engagement has not improved since the age of 15. However, his mood remains stable and there are no signs of agitation. Professor Cullity has no concerns about his dangerous behaviours. Mr Cullity remains with no insight into his psychosis. He believes he only needs to take treatment when he is on a court order.

Professor Cullity confirms there is no hostility at home. However, both his wife and he are worried that Mr Cullity will not keep medication compliance as he is now even with remote supervision. Also, he is not financially independent as he is only on the Disability Support Pension. We suspect that he wants to spend more time on the project by leaving home, similar to what he did in the past. His family is not supportive of his plan.

Discussion with Community Team and Other Documents Review

I discussed this case further with the community team workers. In the last couple of years, Mr Cullity has made no significant progress. He is not interested in rehabilitation programs or activities despite repeated efforts or trials including motivational interviews. He is too busy with his projects and shows no other interests. He has no other physical activities based on his somatic delusions. He has no social contacts. It is difficult and almost impossible to work with him although he appears to be a polite young man.

Opinion and Recommendation

It is clear that Mr Cullity needs ongoing treatment. Over the last few years, his psychosis remains largely unchanged, especially with his delusions, although his behaviours are settled. He is preoccupied with his project and needs to be successful in the computer industry. He is reluctant to participate in active rehabilitation programs and attributes to delusional physical illness. It appears that his psychosis becomes somewhat treatment resistant. He is seeking independence to finish project which causes concerns of mine as this appears to repeat his behaviour in 2013 when he moved out of home and lived in the street before he stabbed others.

Mr Cullity showed little remorse into his past actions. He expresses superficial regrets out of social norm. However, he lacks true understanding about the severity of his behaviour as well as the impact of his illness.

I am not supportive of him moving out of home and suggest that Clozapine needs to be tried. Clozapine is the medication for treatment resistant schizophrenia. He has been tried with a number of antipsychotic medications with limited effect and his compliance still needs to be supervised. I would like to see more change in his overall treatment over the next 12 months.

Thank you again for requesting this report. I hope this report can provide assistance to the Court making decisions and I am happy to be contacted should you wish to clarify any details further.

Yours sincerely,



DR CATHERINE YE
Consultant Psychiatrist

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19/10/17

Dear Dr. Wong

RE: Giles Cullity, DOB: 22/07/1993

I see him again at Tranmere clinic.

His clozapine has been successfully titrated up to 300mg nocte. However, his serum level remains under the therapeutic range. It is difficult to increase further due to side effect of sedation. Therefore his antipsychotic is still boosted with aripiprazole depot 300mg 4 weekly.

He is less delusional and more reactive and interactive. We are planning for independent living and will refer him for OT assessment.

I will increase venlafaxine to 150mg mane. This will increase his morning alertness.

Therefore he will be on

- Clozapine 300mg nocte
- Aripiprazole 300mg 4 weekly imi
- Venlafaxine 150mg mane

I will review him in due course.

Regards,

Dr. Catherine Ye

Senior Consultant Psychiatrist

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10/10/2018

Dear Dr. Wong

RE: Giles Cullity, DOB: 22/07/1993

I see him again at Tranmere clinic.

He remains stable in mental state. Recently he had neuropsychological test and formal report is still pending.

OT report showed that he had some independent living skills. However, he does not want to engage further with our service with support with assistance in terms of gradual transit to independent living. He is not interested in training and activities in community rehabilitation centre.

He has gained a lot of weight. However, he lacks exercises and still takes body building powder for muscle build. I suggest him to see a dietician.

Currently, he is on:

- Clozapine 300mg nocte
- Aripiprazole 300mg 4 weekly imi
- Venlafaxine 150mg mane

Regards,

Dr. Catherine Ye

Senior Consultant Psychiatrist

Government of South Australia | Health
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14/12/2020

RE: Giles Cullity, DOB: 22/07/1993

Mr Cullity is a 27 year old single man who has been residing with his parents. His main source of income is the Disability Support Pension and NDIS. He has been diagnosed with Schizophrenia. It is likely he has a prolonged prodromal psychosis or untreated psychosis before this diagnosis was fully established.

Previously on home detention bail with monitoring device on ankle for attempted murder charge; matter has since finalized with Giles receiving 3 year suspended sentence. Sentence / forensic licence concluded in December 2017. There is a low threshold to apply for a CTO in the event of non-compliance.

Mr Cullity started to contact Mental Health Services when he was in high school. At that time, he presented with depression and suicidal ideation. On reflection, he started to experience unusual thoughts back then. Mr Cullity became increasingly grandiose about his computer project and he was more preoccupied with his so called genetic illness. Meanwhile, his academic performance deteriorated as well as other daily functions including personal hygiene.

He has been admitted to hospital a couple of times and currently is followed up by Eastern Community Mental Health Service, Adelaide. In the past, he was under a Community Treatment Order due to non-compliance issues. He is still under supervision of his parents for oral antipsychotic and antidepressant medication. Mr Cullity has chronic delusions around his computer project and appearance with somatic delusions. The intensity of his delusions fluctuates with treatment.

He appears lacking of insight into his illness and the need for ongoing treatment. Constantly, he debates with the treating clinicians about his diagnosis.

Forensic history:

- Assault police in 2010
- Attempted murder in 2013

Since the first contact with the Mental Health Service, Mr Cullity has been treated with olanzapine, dose varied from 10mg to 15mg. Citalopram was started for depression. Dose range was 30-40mg. In January 2014, non-compliance was reported. Therefore depot was started and CTO L2 was put in place. From February 2014 to September 2015, he was on paliperidone 75mg 4 weekly and the dose was reduced to 50mg until February 2015 when it was further reduced to 25mg. Until February 2016, depot was ceased, risperidone 2mg was started. Due to his somatic complaints, which however, was likely secondary to his somatic delusion, risperidone was changed to aripiprazole. The dose was titrated up to 20mg. He started to complain about akathisia.

Clozapine was started in 2017 with reasonable effects. The intensity of delusion reduced and his function has since improved. However, he is still preoccupied with somatic delusions and taking large amounts of supplements which caused hepatic and renal impairments from time to time.

Currently he is on

- Venlafaxine 150 mg daily
- Clozapine 300mg nocte – level fluctuating
- Aripiprazole 300mg 4 weekly via ECMHS

Developmental and Social History

Mr Cullity is the second of 4 boys. He was born in Scotland and came to Australia with family at the age of eight. He reports no problems in his early life. His father works as a professor of Philosophy of University of Adelaide and his mother is a musical teacher of an elite private school.

He attended St. Ignatius College for primary school. He does not like that school. He went to Marryatville for high school where he was involved in the band. He did not have many friends in school and never had a girlfriend. His grades started to drop in Year 10 when he became obsessive about weight and muscles. He failed subjects in Year 10 and 11 and later some in Year 12. He managed to enter university by doing STAT but failed first year of bridging course.

He became homeless after he finished school in order to finish his "computer project" which was driven by his psychosis. He then ended up with an argument with another homeless person over the ownership of a mattress. He then cut the neck of the other person.

He has no other interests or hobbies, other than his project and body building in his peculiar way.

A+D

No regular drug and alcohol use. Has used THC and alcohol in the past, reported not liking the effects of these substances.

Family history

Mood disorder was reported in his paternal grandfather, a paternal uncle and a maternal uncle.
His younger brother was diagnosed with Bipolar illness and the youngest brother has Asperger's syndrome.

Current mental state is quite stable with no overt psychosis. However, he is still preoccupied with body building, which is rather an overvalued idea. When we discussed his previous assault, he showed minimal remorse towards the victim which is a concern. He is however, willing to continue treatment as a voluntary patient and can see the benefits of the clozapine.

Both parents are very supportive of the treatment and they used to supervise compliance daily.

Father is now working in ACT and family is moving to Canberra. We are now transferring his care to you.

Thank you for your ongoing care for him.

Yours sincerely

Dr Catherine Ye
Senior Consultant Psychiatrist